

Run a 3-track sequenced defense — Defend, Shift, Preserve — with stage-gates at months 3, 6, 12 tied to adjacent-launch traction and biosimilar uptake signals.

\$35–55M redeployment, fully self-funded from established-indication HCP and DTC spend. Adjacent indication launch stays fully funded. Holds 60–70% of patient volume through biosimilar entry — Humira benchmark.

DEFEND

Patient retention — Co-pay + hub + bridge

SHIFT

Commercial weight — Adjacent indication launch

PRESERVE

Loyalty + access — KOL + payer fortifications

Worked example • fictional brand • numbers directional from public benchmarks (Humira, Stelara, Remicade)

Brand defense scenario • produced by the Rapid POV Engine • ~75 minutes from initial brief

A \$1B specialty inflammation biologic enters the most exposed 18 months in its lifecycle.

\$1B

US revenue today

Established specialty position. Loyal prescriber base across rheumatology, dermatology, gastroenterology.

12 mo

biosimilar entry

First biosimilar approval expected within 12 months. Payer step-edit pressure already building in commercial books.

6 mo

adjacent indication

Adjacent indication launches in 6 months. Material upside if defended franchise stays intact through transition.

Three vectors converging in one window. Each is manageable alone. Together they reshape the franchise.

Prescriber loyalty alone won't hold. The switch happens at the pharmacy counter and on the EOB.

THREE PRESSURE POINTS

1

Payer step-edits force a trial of biosimilar before brand.

Patients unwilling to navigate the step lapse — even when the prescriber prefers brand.

2

Patient out-of-pocket diverges sharply at copay reset.

Without an aggressive co-pay program, branded biologic OOP can be 4-6x biosimilar at the counter.

3

Adjacent indication launch competes for HCP attention.

Internal field force has only so many quality calls. Without redeployment design, the new launch starves the defense.

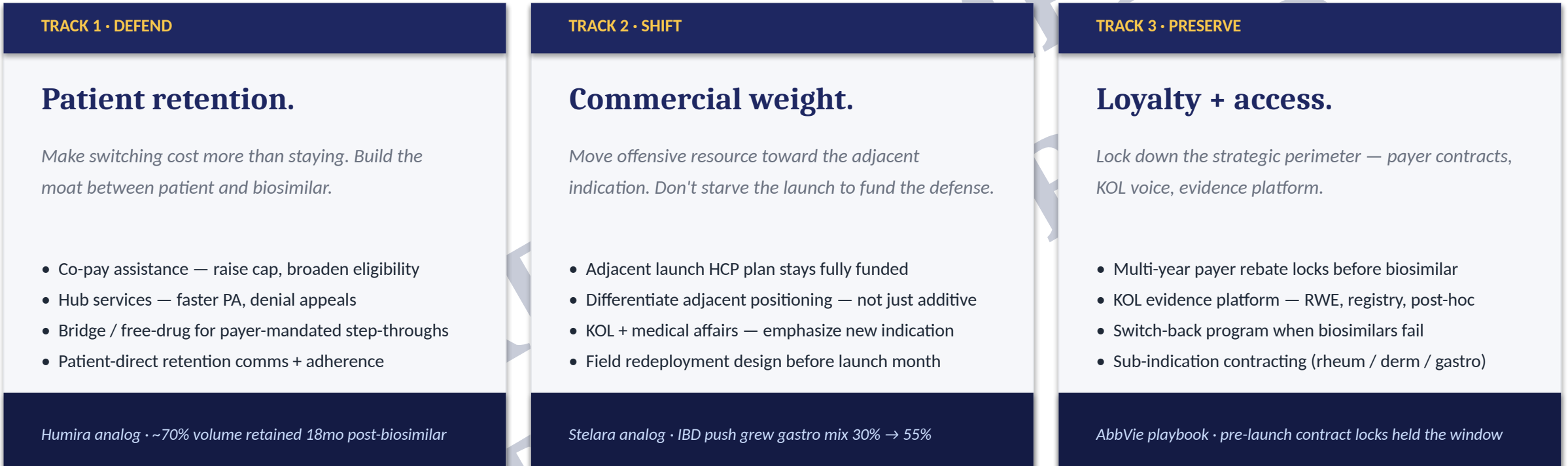
THE REMICADE LESSON

Time is not a defense.

Biosimilars took 7 years to reach 50% of Remicade share — and J&J still lost 80% of US revenue. They underinvested in patient retention. We will not repeat that.

J&J 2016–2023 10-Ks • IQVIA biosimilar tracking

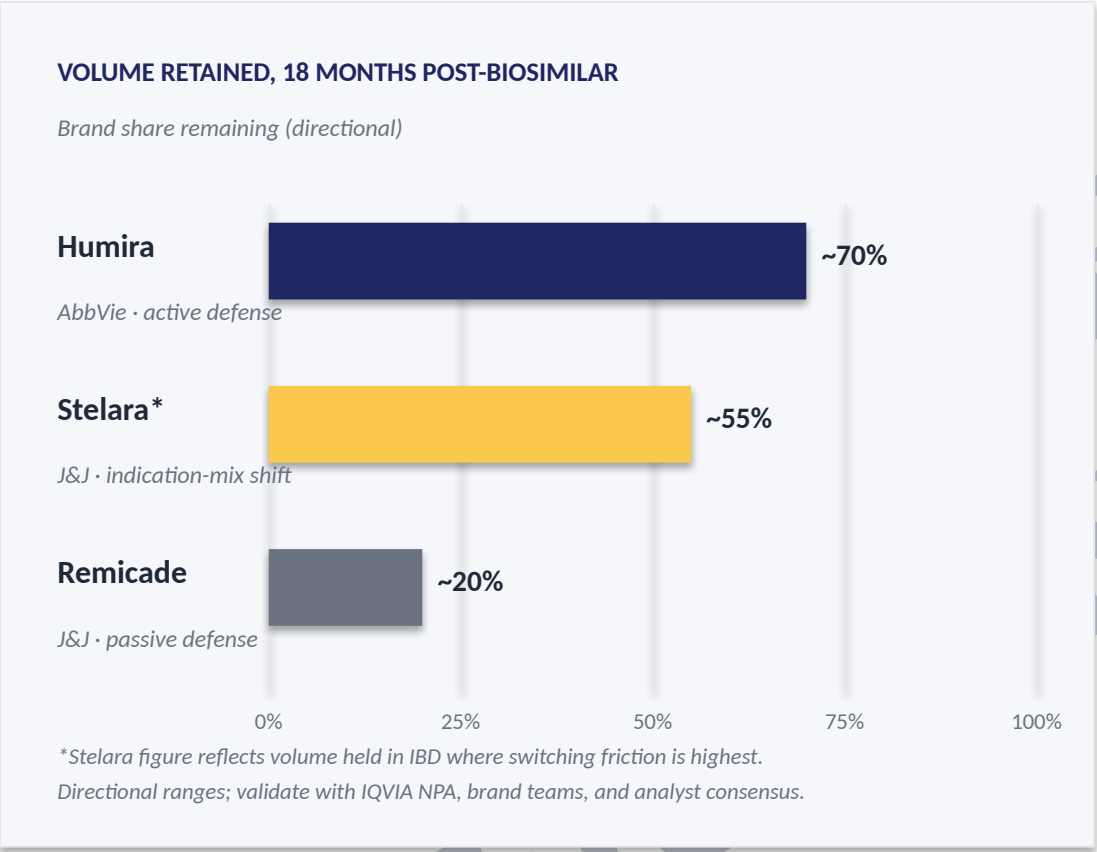
Three tracks running in parallel, sequenced by what each defends and when it activates.



STAGE-GATES



Three precedents tell us where to spend, where to bet, and what it costs to ignore.



WHAT EACH TELLS US

HUMIRA

Patient-side moat is the highest-ROI lever.

Co-pay program + hub + bridge held ~70% of volume 18 months in. Defended ~\$4–5B vs. consensus base case.

STELARA

Push volume into hard-to-switch indications pre-biosimilar.

IBD share of total Stelara grew 30% → 55% by sequenced indication expansions. Adjacent launch is our version of this.

REMICADE

Time alone is not a defense.

7 years to 50% biosimilar share — but \$4.8B → \$0.9B revenue decline. Underinvested in patient retention.

\$35–55M redeployment, fully self-funded. Adjacent launch protected.

↑ WHERE BUDGET GOES — PATIENT SERVICES	↓ WHERE IT COMES FROM — REDEPLOYMENT
Co-pay assistance expansion\$15–35M Raise cap, broaden eligibility, extend duration	HCP promo • established indication–\$20–40M Reduce reach to lapsed/low-decile HCPs
Hub services intensification\$8–15M Nurse navigators, faster PA, denial appeals	DTC • established indication–\$15–30M Mature awareness; marginal dollar buys little
Bridge / free-drug program\$5–12M For payer-mandated step-throughs	Non-critical medical affairs–\$3–8M Cut tail conferences and redundant ad boards
Patient retention comms\$4–8M Adherence app, peer support, MyBrand-style	DO NOT CUT — adjacent launch spend\$0 The launch IS part of the defense (Stelara analog)
PATIENT SERVICES SUBTOTAL\$32–70M	REALLOCATION ENVELOPE–\$38–78M

Three checkpoints. Three things we'll be asked to defend.

MONTH 3

Adjacent launch readiness check

Sales force pivot tested. KOL platform active. Patient-services spend at run-rate. Decision: pull launch trigger or extend prep.

MONTH 6

Adjacent launch + weight shift

First 90-day adjacent NRx and HCP reach vs. plan.
Defense holds $\geq 95\%$ baseline volume. Re-allocate field calls toward new indication.

MONTH 12

Biosimilar entry · re-evaluate mix

Patient retention at 90 days post-biosimilar. If $< 60\%$, escalate co-pay / bridge. If $> 70\%$, scale back bridge spend.

READY TO DEFEND

► The 6-month adjacent-launch window is tight.

We compress prep timeline. Pre-launch field readiness must hit week 16 or we slip and fund Track 1 from reserves.

► Sub-indication granularity (rheum vs. derm vs. gastro).

Tilt patient-services and payer plays toward highest switching friction. Validate with by-specialty IQVIA NPA reads.

► Payer-mandate risk if biosimilar gets exclusive.

If a major PBM goes biosimilar-exclusive, bridge cost runs above range. Pre-modeled: +\$8–15M offset by deeper HCP cut.